



POLICY

Medication Management – Specialised and Community

Scope (Staff):	Medical and Nursing
Scope (Area):	ICAMHS Specialised and Community

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#)

Only use this box if relevant (e.g. clinical document or available on an external website)

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Aim

To guide staff in the appropriate storage, prescribing and administration of medications to Infant, Child and Adolescent Mental Health Service (ICAMHS) patients to ensure optimal care to ICAMHS patients, families and compliance of staff with relevant policies and legislation.

Risk

Non-compliance with legislation and ICAMHS policy may result in adverse patient outcome, loss, theft / diversion or misuse of medications and reputational damage.

Definitions

Specialised Services: for the purpose of this document specialised services in CAMHS include the following clinical services: Swan, Bentley, Armadale, Touchstone and Midland Hub

Authorised Staff: staff employed by CAHS involved in the procurement, distribution, storage, and disposal of medications.

- This includes Medical, Dental, Nursing, Pharmacy and Anaesthetic Technicians as defined by the WA [Medicines and Poisons Regulations 2016](#)¹, [Australian Health Practitioner Regulation Agency](#) (AHPRA)², and the WA [Medicines Handling Policy](#)³.

Controlled Medications: all Schedule 8 (S8) and Restricted Schedule 4 (S4R) medications.

Patient's Own Medication (POMs): any medication that is brought into an ICAMHS clinic by the patient / family.

Restricted Schedule 4 Medication (S4R): Schedule 4 medications subject to additional storage and recording requirements beyond those required for regular Schedule 4 medications.

- Also known as Schedule 4 Recordable medications. A list of these medications can be found in [Risk Based Requirements for Medicines Handling](#)⁴ (see [Medicines Handling Policy](#)³)

Schedule 4 Monitored Medicines (S4 Reportable Poisons): medications that are classified as drugs of addiction¹.

- Are considered to have a higher risk of abuse, misuse and physical or psychological dependence. Listed in Schedule 6 of the WA [Medicines and Poisons Regulations 2016](#)¹.

Schedule 8 Medication (S8): drugs of addiction.

- This category is for substances that should be available for use but require restriction of manufacture, supply, distribution, possession and use to reduce abuse, misuse and physical or psychological dependence. For further information refer to [The Poisons Standard \(the SUSMP\)](#)¹¹.

Principles

- Medicines must be handled in accordance with the [Medicines and Poisons Act 2014](#)⁵, WA Health [Medicines Handling Policy](#)³ and local CAMHS policy. Any use of medicines outside lawful practice will breach legislation and serious penalties may apply.
- Using a medicine for personal reasons, where not part of the lawful treatment of a patient under the care of CAHS is both theft and serious misconduct that may result in disciplinary action.
 - Refer to the CAHS [Management of Notifiable and Reportable Conduct](#) procedure.
- Other legislation and penalties for unethical misconduct or practicing while impaired (due to drug use) may also apply, such as the [Health Practitioner Regulation National Law \(WA\) Act 2010](#)⁶.

Procurement, Distribution, Storage and Disposal

Community ICAMHS

- Community ICAMHS sites must not procure or store any medicines, except for the following Schedule 3 emergency medications:
 - Salbutamol MDI 100 microgram/actuation
 - Adrenaline (epinephrine) 150 microgram/0.3 mL prefilled syringe (autoinjector junior)
 - Adrenaline (epinephrine) 300 microgram/0.3 mL prefilled syringe (autoinjector)
- A maximum of 3 units of each of the above medications can be kept on site.
- It is the responsibility of the manager (Senior Registered Nurse (RN) / authorised staff member) to procure medicines (e.g. for emergency situations) from a nominated community pharmacy.
- Drug samples and starter packs must **NOT** be accepted from pharmaceutical sponsors.
- Procurement of medications must be in line with the [WA Paediatric Statewide Medicines Formulary](#) (SMF)⁷.
- Once procured, medications must be:
 - stored in an area that only permits access to authorised staff (i.e. a locked cupboard or room not accessible to the public)

- kept in their original packaging
- stored in accordance with the manufacturer's storage recommendations
- checked for expiry at least every 3 months (refer to [Appendix 4: Expiry Check Form for Emergency Medications](#)).
- Expired medications which have been procured should be disposed by taking them to a nominated community pharmacy

Specialised ICAMHS

- Specialised ICAMHS sites may be required to purchase and store emergency medicines which are unscheduled or included in Schedule 2 or 3 of the [Poisons Standard](#)¹¹. Specialised ICAMHS sites may not be serviced by a public hospital Pharmacy Department, however the Medicines and Poisons Act and Regulations make provision for these medicines to be purchased from a community pharmacy without a prescription.
 - Specialised and Community ICAMHS sites must not procure Schedule 8 (S8) or Schedule 4 Medicines (S4) for imprest.
- Only small quantities of Schedule 2 and Schedule 3 medications in quantities consistent with personal use may be procured.
- POMs clinically relevant to the management of the patient may be accepted for storage and administration.
- It is the responsibility of the manager (Senior Registered Nurse (RN) / authorised staff member) to procure medicines (e.g. for emergency situations) either via a nominated community pharmacy or Perth Children's Hospital (PCH) Pharmacy Department if an agreement is in place.
- Specialised ICAMHS sites may procure the following emergency medications from PCH Pharmacy Department on request:
 - Salbutamol MDI 100 microgram/actuation
 - Adrenaline (epinephrine) 150 microgram/0.3 mL prefilled syringe (autoinjector junior)
 - Adrenaline (epinephrine) 300 microgram/0.3 mL prefilled syringe (autoinjector)
 - Glucagon 1 unit (1 mg) kit
- Drug samples and starter packs must **NOT** be accepted from pharmaceutical sponsors.
- Procurement of medications must be in line with the [WA Paediatric Statewide Medicines Formulary](#) (SMF)⁷.
- Once procured, medications must be:

- stored in an area that only permits access to authorised staff (i.e. a locked cupboard or room not accessible to the public)
- kept in their original packaging
- stored in accordance with the manufacturer's storage recommendations
- checked for expiry at least every 3 months (refer to [Appendix 4: Expiry Check Form for Emergency Medications](#)).
- Keys for the S8 and S4R safe must be kept on the person of the manager (Senior RN / authorised staff member) when the clinic has taken receipt of an S8 or S4R POM.
 - At all other times the keys must be locked in a separate safe to ensure chain of custody, ideally with swipe card access or a combination safe.
 - A log of date and time of when an authorised staff member has removed and returned the keys must be maintained and available for auditing for 2 years.
 - Refer to the [Medication Safe Key Handling Procedure](#).
- Expired medications which have been procured should be disposed by either taking them to a nominated community pharmacy or the PCH Pharmacy Department.
 - POMs requiring disposal should be returned to the family / carer with instruction to take to a nominated community pharmacy for destruction.

Prescribing – Authorisation, Approval and Governance

- All staff who prescribe must do so within credentialed scope of practice as per CAHS [Credentialing and Recredentialing of Medical and Dental Practitioners Policy](#).
- A Best Possible Medication History (BPMH) including a list of adverse drug reactions and allergies (as per 'verification of medications on admission or start of an episode of care' in the [Medication Safety Policy](#)) must be documented at the time of admission to, and discharge from the service.
 - Ideally two sources of information should be used to ascertain a BPMH.
- BPMH must be recorded in the patients' medical record with documentation of sources used to confirm the history.
- Prescriptions must be compliant with the [Western Australia Paediatric Statewide Medicines Formulary](#) (SMF)⁷.
 - Requests to prescribe outside the SMF require prior approval from the CAHS Drugs and Therapeutics Committee (DTC) via an Individual Patient Approval (IPA) (Refer to the [Prescribing – Authorisation, Approval and Governance Policy](#)).

- Allergy and adverse drug reaction (ADR) information must be recorded in the following:
 - Best Practice® software
 - WA paediatric Hospital Medication Chart (pHMC) if medication is administered onsite (e.g. Specialised ICAMHS)
 - Patient's health / medical record
 - Refer to the [Allergy and Adverse Drug Reaction Management Policy](#)
- Newly identified Allergies and adverse drug reactions (ADRs) must be recorded in the online [Adverse Drug Reaction Form](#) (refer to [CAMHS Clinical Alerts Procedure](#)).
- Consent must be obtained and documented for all prescribing using the [Consent for Medication Use Form](#) (MR505.00/MR859.01).
 - For off-label prescribing, refer to [Use Of Medications For Other Than Manufacturers Approved Indications Policy](#) and the [Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians Guideline](#).
- Where relevant for Aboriginal and Torres Strait Islander Patients, decisions to initiate new medications must be discussed with the patient's Aboriginal Mental Health Worker to discuss any spiritual impacts of new diagnoses or medication prescribing.
- Where relevant, the CAHS [Children's Antimicrobial Management Program \(ChAMP\)](#) restrictions must be followed for antibiotic prescribing (refer to the [Children's Antimicrobial Management Program \(ChAMP\) Policy](#)).
- All prescriptions must be generated electronically via Best Practice® software.
- Prescriptions for administration onsite to a patient (e.g. Specialised ICAMHS) must be prescribed on the WA pHMC.
- The prescribing of Schedule 8 stimulant medication is restricted in Western Australia.
 - Prescribers must apply to the WA department of Health for approval and be compliant with the [Monitored Medicines Prescribing Code](#)⁹.

Medication Counselling

- Patients, family, carers and guardians must be provided with verbal and written information as part of medication counselling when new medications are prescribed.
- For Aboriginal and Torres Strait Islander patients, counselling must occur in collaboration with the patient's Aboriginal Mental Health Worker and Case Manager

- Written information provided to patients may be Consumer Medicines information (CMI) handouts provided by the manufacturer or CAHS endorsed written information e.g. [Health Facts](#) or [Choice and Medication Printable Leaflets](#).
 - When counselling Aboriginal and Torres Strait Islander patients, consider using Keeping our Mob Healthy handouts where they are available.

Medication Preparation, Checking and Administration

- Specialised ICAMHS sites may need to administer a small amount of emergency medicines as required as well as oral 'prn' (when required) POMs. Community ICAMHS sites may need to administer an emergency medication as required.
- Preparation, administration and checking of medications including following the 6 rights of safe medication administration must be compliant with the PCH [Medication Preparation, Checking and Administration Procedure](#) – specifically appendix 1).

Six rights of safe medication administration

1. Right patient (and chart)	4. Right route
2. Right drug	5. Right time
3. Right dose	6. Right documentation

Independent Second Check

- The process for an independent second check requires a second authorised staff member to:
 - independently check the six rights without cues from the person doing the initial check,
 - witness the preparation of the medication,
 - confirm the final dose to be administered to the patient.
- If there are discrepancies in the dose / volume calculations both staff members are to recheck their calculations. If a discrepancy remains, then another senior staff member is to be consulted.
- Registered Nurses (RNs), Medical Practitioners, Pharmacists and / or any Medication Competent Enrolled Nurses (ENs) are authorised to independently second check a medication.
 - Undergraduate Nursing Students refer to the PCH [Medication Administration Policy](#) with reference to Appendix 1 – ‘Summary of Medication Administration Restrictions by Staff Designation’).

- When an independent second check process is being undertaken by two nursing staff, one staff member must be a RN.
- Both staff members are equally responsible for ensuring the independent second check process is followed.
- Both staff members must attend to the patient to confirm the right patient is administered the medicine.
- Independent second checking must be performed when administering the following groups of medications (refer to the [Medication Preparation, Checking and Administration Procedure](#) for the full list):
 - High risk medicines (refer to [High Risk Medicines Policy](#)).
 - S8 and S4R medications,
 - Anticonvulsant and sedative medications,
 - Any medication which is unfamiliar to the staff member administering it.

Procedure for administration

- Perform hand hygiene prior to preparing and administering any medication (refer to [Hand Hygiene Policy](#)).
- Check the medication order to ensure it is legal, legible, complete and uses only approved abbreviations (refer to [Abbreviations for Clinical Documentation Policy](#)).
- Every medication order must include generic drug name, dose, drug form, route, frequency or directions.
- The allergy section of the medication chart must be completed before the chart can be used to administer any medication (refer to [Allergy and Adverse Drug Reaction \(ADR\) Documentation and Reporting Procedure](#)).
- Medications must never be administered if there is any doubt regarding drug orders, including illegible or incomplete orders.
 - Raise concerns with the prescriber and / or senior nursing staff.
- Incorrect or illegible orders must be ceased by the prescriber and rewritten in full.
 - Refer to the PCH [Medication Preparation, Checking and Administration Procedure](#).
 - See Guidelines and Resources referred to by the WA Health [Medication Chart Policy](#)¹⁰.

Patients' Own Medications

- POMs must be stored in a secure manner to prevent misadventure and loss of medications.

- They are not to be left with the patient or parent / carer and must be stored securely and identifiable as belonging to the patient. Additional Storage requirements are mandated for Patients' Own S8 and S4R medications. Clinics may choose to have the patients' family sign for quantity received and returned to prevent disputes using [Appendix 3: Patients' Own Medication Receipt and Return Form](#) if required. This should be stored until POMs are returned to the parent / carer.
 - Refer to [Appendix 1: Patients' Own Medication Storage Procedure](#)
 - Refer to [Appendix 2: Patients' Own Medication Administration Procedure](#)
 - Refer to [Appendix 3: Patients' Own Medication Receipt and Return Form](#)

Record Keeping and Discrepancy Management

- POMs must always be returned to the patients' parent / carer once they are discharge from the service at the end of a clinic.
- Identified discrepancies in any S8 and S4Rs must be investigated immediately and any loss or unresolved discrepancy reported via the [Medicines Discrepancy Report Form](#) (refer to [Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy](#)¹²). Further advice refer to the [CAHS Controlled Medicines Governance Framework](#) for advice.
- Daily safe count checks must be completed when patients' own S4R and S8s are on site.
- Completed S8 and S4R registers must be stored for five (5) years from their completion and be available in the event of audit or investigation.

Clinical Incidents

- Any clinical incident which could have or did lead to unintended and / or unnecessary harm to a patient must be reported (Refer to the [Clinical Incident Management Guideline](#)).

Compliance and monitoring

- Compliance against this policy will be measured by conducting the ICAMHS Medication Management Audit and ICAMHS Prescribing Audit as per the Community CAHS Audit Schedule.

References and related external legislation, policies, and guidelines
1. WA Medicines and Poisons Regulations 2016
2. Australian Health Practitioner Regulation Agency (AHPRA)
3. DoH MP139/20 Medicines Handling Policy

4. DoH Risk Based Requirements for Medicines Handling Including requirements for Schedule 4 Restricted medicines
5. Medicines and Poisons Act 2014
6. Health Practitioner Regulation National Law (WA) Act 2010.
7. Western Australia Paediatric Statewide Medicines Formulary (SMF)
8. Federal Register of Legislation Poisons Standard September 2021
9. DoH Monitored Medicines Prescribing Code , December 2024. Medicines and Poisons Regulations 2016.
10. DoH MP0078/18 Medication Chart Policy
11. DoH and Aged Care The Poisons Standard (the SUSMP) Therapeutic Goods Administration (TGA), 27 February 2025.
12. DoH MP 0103/19 Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy and the Medicines Discrepancy Report Form

Related CAHS internal policies, procedures and guidelines
CAHS Management of Notifiable and Reportable Conduct Procedure
CAMHS Medication Safe Key Handling Procedure
CAHS Credentialing and Recredentialing of Medical and Dental Practitioners Policy
CAHS Medication Safety Policy
CAHS Prescribing – Authorisation, Approval and Governance Policy
CAHS Allergy and Adverse Drug Reaction Management Policy
CAMHS Clinical Alerts Procedure
CAHS Use Of Medications For Other Than Manufacturers Approved Indications Policy
CAMHS Off-Label Psychotropic Prescribing Guide for CAMHS Clinicians Guideline
CAHS Children's Antimicrobial Management Program (ChAMP) Policy

CAHS Children's Antimicrobial Management Program (ChAMP)
PCH Medication Preparation, Checking and Administration Procedure
PCH Medication Administration Policy
CAHS High Risk Medicines Policy
CAHS Hand Hygiene Policy
CAHS Abbreviations for Clinical Documentation Policy
PCH Allergy and Adverse Drug Reaction (ADR) Documentation and Reporting Procedure
CAHS Procurement, Distribution, Storage and Disposal of Medication Policy
CAHS Clinical Incident Management Guideline

Useful resources (including related forms)
CAHS Adverse Drug Reactions Form
CAHS Consent for Medication Use Form (MR505.00/MR859.01)
CAHS Health Facts
CAHS Choice and Medication Printable Leaflets
DoH MP 0103/19 Medicines Discrepancy Report Form (Reporting of S4R and S8 Medicine Discrepancies Policy)
CAHS Controlled Medicines Governance Framework

This document can be made available in alternative formats on request.

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Healthy kids, healthy communities

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Appendix 1: Patients' Own Medication Storage Procedure

- All POMs should be stored in a manner that clearly identifies them as belonging to the patient and separates them from other patients' medications / property.
- POMs should only be handled by authorised staff and should be stored securely and separately from clinic ward stock (if relevant) in a designated secure storage receptacle. The storage receptacle must be kept locked when not in use by staff. If a cupboard is used, the cupboard must be securely attached to the floor or wall.
- S4R and S8 POMs must be stored securely in a safe with the same level of security as other S4R and S8 medicines:
 - In patient care areas, all S4R/S8 POM medications are to be signed into a hard-copy register (HA212 register) by two authorised persons. Two (2) authorised staff must count the amount of POM being stored, with the quantity released to the clinic acknowledged by the family.
 - A separate register is required for S8 and S4R Medications.
 - There should be one patient per page in the register and each page should include information that uniquely identifies the patient with certainty (preferably includes patient name, Unit Medical Record Number (UMRN) and date of birth).
 - The name, strength, form, and quantity of S8/S4R POM medication is to be recorded.
 - The date and time of transactions are to be recorded (refer to [Procurement, Distribution, Storage and Disposal of Medication Policy](#)).

Refer to [The Poisons Standard \(the SUSMP\)](#)¹¹ and WA [Medicines Handling Policy](#) for list of S8 and S4R Medications.

- Safe commissioning and decommissioning (Specialised ICAMHS sites):
 - The safe may be decommissioned when nil S4R and S8 POMs are being kept onsite. During this time the safe may be left open with nothing inside.
 - A final checked and correct balance is performed in the register of Drugs of Addiction (HA212 register) and the Safe Commissioning/Decommissioning Log in the register is completed by two staff members.
 - A 'NOT IN USE' sign should be placed on the safe.
 - Daily balance checks are not required to be performed during periods of decommissioning.
 - The safe can be recommissioned upon receipt of patient's own S4R and S8 medication. The sign is removed, and the safe commissioning log is completed by two staff members to indicate that the safe is again in use. The S4R or S8 POMs are entered into the register and the safe is then closed and locked with the POMs inside.

- Returning of POM S8 and S4R medications to the patient's parent / carer must be documented in the relevant register as a transaction.
 - If POM S8 / S4R has been used, this must be explained to the family upon return.

Appendix 2: Patients' Own Medication Administration Procedure

- POM can only be administered if ordered by an authorised prescriber using the WA pHMC.
- For POM S8 and S4Rs, details of the administered dose are to be documented in the appropriate register at the time of administration.
- Two authorised staff must be involved in the administration of the POM S8 or S4R.
- Any unusable amount of S8 or S4R medicine remaining after administration of the dose must be discarded in a manner that results in the discard being unrecognisable, unusable, and irretrievable ideally into a pharmaceutical waste container. This disposal should also be recorded in the appropriate register
 - Refer to the [Procurement, Distribution, Storage and Disposal of Medication Policy](#)).

Appendix 3: Patient's Own Medication Receipt and Return Form

Date:	Patient addressograph Label
Medication Receipt	
Medication name:	
Strength:	
Quantity Received:	
Family member name and signature	
Staff Member 1 name and signature	
Staff Member 2 name and signature	

Medication Return	
Medication name:	
Strength:	
Number of doses used:	
Quantity Returned:	
Family member name and signature	
Staff Member 1 name and signature	
Staff Member 2 name and signature	

Appendix 4: Expiry Check Form for Emergency Medications

[illegible]